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Mental Health as an Afterthought? Addressing how Global Health Governance has Failed Conflict-Affected Sudan

Global Health Challenges and Governance
Global Health Commentary
Master of Public Health
CID: 06031009

Word Count: 1,499

Summary Box

- Despite widespread psychological trauma, global mental health responses in (North) Sudan remain underfunded, undercoordinated, and mismatched to local needs.
- Current global governance frameworks often prioritise imported Western-centric mental health models over more context-sensitive, community-led approaches.
- This failure to meaningfully involve local actors and knowledge systems into decision-making reflects deep and historic power imbalances in global health governance.
- Reimagining global mental health governance to prioritise localisation, equity, and sustained investment is crucial to long-term recovery in Sudan and other such context-sensitive settings.

1.0 Introduction

As (North) Sudan endures another wave of widespread and violent conflict, the physical toll in the form of bombed hospitals, fractured families, and displaced millions is visible.[1,2] Yet, beneath the rubble lies a quieter crisis: Widespread and escalating psychological trauma.[2] Mental health has long been a neglected aspect of humanitarian health intervention, and Sudan exemplifies the heavy consequences of this oversight.[2,3] Mental health, defined by the World Health Organisation (WHO) as "a state of mental wellbeing in which the individual realizes his or her own abilities, can cope with the normal stresses of life, can work productively, and is able to contribute to his or her community", is seldom prioritised in emergency response.[3]

Global health governance – the structures, institutions, and processes by which global actors influence health policy and outcomes – plays a decisive role in shaping how mental health is addressed in such conflict settings. Trancending national boundaries, it involves multilateral institutions such as the WHO, bilateral donors, and a wide variety of non-state actors (Appendix). However, these governing mechanisms often export standardised frameworks designed in high-income countries (HICs) into vastly different socio-cultural and political environments.[4] This commentary argues that in Sudan, current global mental health governance mechanisms have failed to adequately support effective, equitable, and sustainable mental health care. Imported frameworks, coupled with inefficient funding and exclusion of local perspectives, have rendered mental health an afterthought in Sudan's conflict response.

2.0 Review

Sudan has faced decades of political instability punctuated by episodes of violent conflict, mass displacement, and economic collapse.[1] Since the outbreak of civil war in April of 2023 between rival military factions, over 14.6 million people have been displaced both internally and across borders.[1] Adding to this concerning figure, 30.4 million people – more than half Sudan's population – have been left in need of humanitarian support.[1] Consequently, the toll on mental health is immense: Trauma stemming from violence, loss of loved ones, family separation, and persistent insecurity compound to generate widespread psychological distress.[2] However, Sudan's health system – already fragile before the war – has dilapidated in many areas, leaving a concerning vacuum in both physical and mental health services.[2]

Global frameworks such as the WHO's *Mental Health Gap Action Programme (mhGAP)* and the *Inter-Agency Standing Committee (IASC) Guidelines on Mental Health and Psychosocial Support (MHPSS) in Emergency Settings* have sought to address mental health in such crisis contexts.[5,6] These initiatives promote task-shifting, integration of mental health into primary care, and community-based psychosocial support. Yet, their implementation in frangible states such as Sudan often face significant barriers and frequently lack contextual adaptation.[7] Standardised toolkits are deployed without sufficient regard for potential cultural or structural mismatches, including in beliefs, health system infrastructure, or local priorities.[7]

Critical theoretical and systematic reviews of mhGAP have elucidated that while its evidence-base is indeed robust, the framework tends to over-emphasise clinical approaches rooted in Western psychiatric models, which often prioritise diagnostic categorisation and pharmacological treatment over holistic or more community-based ideas of mental wellbeing.[8,9] For instance, work by Ventevogel *et al*. critiques the dominance of 'biomedical framing' in humanitarian mental health responses, calling for greater cultural sensitivity and community ownership.[10] Moreover, local practitioners and researchers are often excluded from decision-making processes, perpetuating an ongoing cycle of dependency and misalignment.[8] The result is a landscape where mental health services – if existent at all – are fragmented, underfunded, and perceived by foreign or irrelevant by the communities which they are meant to serve.[8,10]

A comparative example of this can be found in Syria, where global mental health initiatives were imported with minimal contextualisation.[11] Work by Hassan *et al*. found that international non-governmental organisations (NGOs) largely sidelined Syrian mental health professionals and traditional coping mechanisms, resulting in low uptake of services, and resistance within affected communities.[11] These parallels underscore the risks of such one-size-fits-all governance models.

3.0 Critical Evaluation

As such, this commentary contends that current global mental health governance frameworks have failed in Sudan because they are insufficiently funded, under-adapted to local contexts, and exclude integral local voices. It argues that these failures are not simply logistical, but reflect deeper issues of power, equity, and epidemiology in global health.

3.1 Economic Inadequacies

First, mental health remains chronically underfunded in global health budgets.[3,12] According to the WHO's Mental Health ATLAS of 2020, median government mental health expenditure in low-income countries (LICs) is less than $1 USD per capita annually.[12] What's worse, Sudan's own health budget has been gutted by the conflict, and international humanitarian aid has not filled the gap.[1,2] Despite widespread psychosocial needs, mental health accounts for less than 2% of all health-related funding in recent humanitarian response plans – a lack of investment which signals that mental health is not viewed as an essential component of health, but rather as an auxiliary concern.[2] Notably, the economic consequences of this inaction are also significant. A widely cited study published by The Lancet Psychiatry estimates that depression and anxiety disorders alone cost the global economy over $1 trillion USD each year in lost productivity.[13] In fragile contexts such as Sudan, untreated mental disorders contribute to diminishing educational attainment, truncated workforces, and other potentially complex and wicked long-term public health burdens.[2] The neglect of mental health, therefore, is not only a humanitarian failure, but an economically shortsighted decision.

3.2 Cultural Mismatch

Second, imported frameworks often fail to resonate with local realities.[8,9] The mhGAP intervention guide, for instance, prescribes structured algorithms and protocols for diagnosing and managing mental disorders but offers little guidance in the way of socio-cultural adaptation.[8,10] In Sudan, where communal healing practices, pseudoscientific care, and traditional healers play central roles in mental wellbeing, a strictly biomedical model is unlikely to gain traction.[9] Reoccurring themes from qualitative studies included in systematic review have shown that failure to engage these local systems results in low service uptake and systemic mistrust.[9] Although local initiatives have emerged to fill these gaps, they often go unsupported. The Amal Centre for Rehabilitation of Victims of Torture in Khartoum (the capital), for example, integrates medical, psychosocial, and community-based approaches to support survivors of trauma.[14] While robust comparative evaluations of their model against the West's biomedical remain scarce, the Centre's accentuation on cultural sensitivity, community dialogue, and holistic support highlights the potential of locally rooted initiatives to address mental health needs through means which resonate with lived Sudanese realities.[14] Yet, such initiatives

seldom receive international funding or recognition, as they fall outside of the dominant models promoted by global governance bodies.

3.3 Colonial Governance

Third, and arguably most importantly, global governance structures have largely marginalised local actors.[4] Decision-making about which interventions to prioritise, fund, or scale is often made by centralised and powerful entities such as the WHO, the United Nations Office for the Co-ordination of Humanitarian Affairs (UN OCHA), and the European Civil Protection and Humanitarian Aid Operations (ECHO) at the international level.[4] These bodies tend to rely on external consultants or partner NGOs based in the Global North, limiting input from on-the-ground Sudanese professionals and communities.[4] This exclusion is both epistemic and practical, undermining the legitimacy of interventions, and perpetuating the colonial dynamics of knowledge synthesis within global health.[4] In effect, local actors face barriers to prioritisation and influence over agenda-setting, relegating them to the role of implementers rather than active decision-makers. As argued by Abimbola *et al.*, genuine decolonisation of global health will require the reimagining governance structures to centre the voices and leadership of those most affected.[15]

3.4 Viewpoints and Limitations

Nevertheless, it is worth acknowledging potential limitations to this commentary's argument. Localising mental health responses is not without its challenges, including variations in service capacity across regions, risks of reinforcing local power imbalances, and difficulties in ensuring a standardised quality of care. Data-collection in conflict-affected settings such as Sudan poses additional challenge, constraining comprehensive health needs assessments and evidence-based planning. The utility of global frameworks such as the mhGAP and IASC is that such guidelines provide scaffolding for response in these resource-constrained settings that are grounded in rigorous evidence-based practices; and the caveat is that without meaningful adaptation and participatory governance, even the best frameworks run the risk of becoming inept or even counterproductive.

4.0 Conclusion

In conclusion, the mental health crisis in (North) Sudan is not merely a byproduct of war – it is a consequence of systematic neglect rooted in the structure of global health governance.[4] Current approaches have prioritised formularised solutions over contextual understanding, marginalising local actors, and perpetuating power imbalances that render mental health an afterthought in emergency response.[2,3] Evidence suggests that a more effective alternative would involve co-producing

interventions with affected communities, integrating local cultural perspectives, and ensuring sustained investment in locally led systems of care.[4] To redress these failures, global mental health governance must be reimagined, and funding must match the scale of need. Frameworks should be co-developed with local stakeholders and flexibly adapted to cultural and systemic realities.[15] And most importantly, governance structures must shift from paternalistic models to participatory, equitable partnerships.[4] Only then can mental health be addressed effectively as a core component of health and human dignity in Sudan and beyond.

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Appendix


Global Policy Arenas
Global Health System
Philanthropies
Civil society organizations
WHO
Multilateral organizations
Hybrid partnerships
Countries
Academic institutions
Private sector
Bilateral cooperation agencies
Media
Trade and investment
Security
Agriculture
Environment
Migration
Education